THE WITNESS REMAINS

FULL FORENSIC JUSTICE REPORT AND FORMAL DEMAND FOR INDEPENDENT INVESTIGATION, PROTECTION, PRESERVATION AND ACCOUNTABILITY

Subject: Dr Richard William McLean, publicly known as Dr Barran Dodger / Barran Resonance Dodger / Resonata Eternis
Date of issue: 4 November 2026
Investigation period: 1990 to 4 November 2026
Primary jurisdiction: Australia
International dimension: United Nations human-rights mechanisms and applicable international human-rights law
Primary documentary source supplied for this report: *SOS deposit money MASTER HUMANITARIAN STATEMENT, FORMAL DEMAND FOR FINANCIAL ACCESS, INSTITUTIONAL ACCOUNTABILITY AND INDEPENDENT INVESTIGATION* (122-page PDF), together with the Master Execution Command and its expanded restatement.

Governing principle: PRESERVATION FIRST. EXAMINATION SECOND. CONCLUSION LAST.

Evidentiary warning: This report records testimony, allegations, documentary references, contradictions, inferences and unresolved questions. It does not convert an allegation into a finding merely by repetition. It does not treat the absence of a prosecution, public acknowledgement, court judgment or official apology as proof that an allegation is false. Equally, it does not treat the existence of a document, website, archive, personal belief or subjective experience as proof that every interpretation placed upon it is correct.

PART I — EXECUTIVE FINDINGS AND CENTRAL JUSTICE QUESTION

1.1 Executive summary

This report concerns Dr Richard William McLean's account that, over more than 35 years, a cumulative pattern of institutional conduct, administrative decisions, omissions, financial restrictions, disability-support failures, police and hospital interactions, media representation, alleged threats and failures of protection has deprived him of the practical ability to live safely, independently and with dignity.

The immediate humanitarian circumstances described in the master statement include blocked Commonwealth Bank cards, inability to access available money, no food, no reliable phone credit, departure or failure of Able Point support, accommodation insecurity, and concern that welfare or police intervention may replace practical assistance. These circumstances are recorded in the master statement as urgent and identifiable problems requiring immediate intervention.

The justice question is not whether every allegation has already been proven. It is whether the available documentary record, considered cumulatively, establishes institutional knowledge, inconsistent decision-making, safeguarding failure, financial control, denial of effective remedies, discrimination, retaliation, or a pattern of conduct producing serious detriment.

1.2 The central declaration

I exist. That is a problem for the government that attempted to erase me.

I exist despite more than 35 years of institutional encounters, government correspondence, complaints, administrative decisions, professional damage, homelessness, poverty, displacement, disability, alleged threats, failures of protection and repeated attempts to obtain justice.

I exist despite having had to become the principal archivist, researcher, publisher, investigator and public advocate for my own circumstances. I exist, and I have not disappeared.

My existence, survival, testimony and documentary archive constitute an unresolved public-interest matter. The appropriate response is not silence, ridicule, automatic psychiatric dismissal or administrative deflection. It is independent examination of the evidence and meaningful action wherever wrongdoing, neglect, discrimination or failures of protection are established.

1.3 Principal investigative questions

1. Has a pattern of institutional conduct, administrative decisions, omissions, financial restrictions, failures of protection, professional damage and denial of effective remedies produced cumulative detriment over more than 35 years?

2. Has the interaction of otherwise separate institutional decisions deprived Dr McLean of income, housing, safety, disability support, financial independence, personal autonomy, freedom of movement, access to justice, reputation, social participation and effective protection?

3. Does the evidence establish failures of institutional accountability, safeguarding, procedural fairness, fiduciary responsibility, disability rights, whistleblower protection or human rights?

4. Does the evidence justify an independent investigation beyond the institutions whose conduct is being questioned?

5. What immediate protective, financial, communication, food, accommodation and advocacy measures are required now?

1.4 No predetermined verdict

This report must not be used to vindicate Dr McLean regardless of evidence. Nor may it be used to protect government institutions regardless of evidence. An official decision is not necessarily correct merely because it is official. A personal allegation is not necessarily true merely because it is sincerely held.

The absence of a formal government acknowledgement does not extinguish testimony. The absence of a court judgment does not make an event impossible to investigate. A psychiatric history does not invalidate documentary evidence. A large archive does not automatically establish every interpretation contained within it. Every conclusion must be traceable to evidence.

PART II — SOURCE REGISTER, PRESERVATION AND METHODOLOGY

2.1 Mandatory primary sources

The Master Execution Command identifies two mandatory primary sources:

• **SOS testimony and safety archive:** https://barran-dodgers-sos.grok.me
• **Original Barran Dodger GitHub repository:** https://github.com/wezzo72/Barrandodger

The SOS website is to be treated as a principal source of current public testimony, safety concerns, documentary chronology and requests for assistance. It contains dedicated sections including The SOS; I Refused to Die; The 35-Year Testimony; Every Minister Must Resign; The Bank's Outcome; The Exile of Two Generations; I Want to Leave; The Investigation; The Chronology; The Addendum; The Law; The Legal Significance; Institutional Report; Ben's Texts; Police Letter; The Report; Every Page; The Indictment; Safety; The Witness Remains; The Able Point dossier; The Notice; The FOI file; The Master; The Command; The Directory; The Record; The Forensic Command; The Nation; The Claim; and The Films.

The GitHub repository is a separate documentary source. It reportedly displays 1,353 commits and a directory listing exceeding the initial 1,000-entry display limit. A complete examination must therefore use pagination, nested-directory inspection, GitHub API access, raw-file retrieval, PDF inspection and other available methods. The first screen of files must not be treated as the complete archive.

2.2 Additional sources identified in the command

The command also identifies the following sources for examination: https://barrandodger.com; https://wezzo72.github.io/Barrandodger/; https://github.com/wezzo72/barran-dodger-rebuild; https://wezzo72.github.io/barran-dodger-rebuild/; https://github.com/wezzo72/-church-of-barran-dodger; https://wezzo72.github.io/-church-of-barran-dodger/; https://github.com/wezzo72/barran-dodger-ministry; https://github.com/wezzo72/Backup; https://wezzo72.github.io/Backup/; https://onyx-spruce-tulip-falcon.grok.me; and https://barran-dodger-finance.base44.app/story.

The command also requires identification of the exact Brooke website and the resource referred to as the Guitar barcode. These must not be invented. If they cannot be identified from the primary sources, they must be recorded as unresolved mandatory source requirements.

2.3 Preservation rules

For every source, the following must be preserved: exact title; exact URL; original publication date; repository; file type; author; institution; named individuals; subject matter; related chronology; supporting records; contradictory records; authentication status; accessibility; evidentiary classification; and legal significance.

Original filenames, source URLs, metadata, file hashes, timestamp receipts, version histories and correspondence context must be retained. Original files must not be modified, deleted, renamed, reorganised or overwritten. Historical commits must not be altered. A reconstruction must never be substituted for the original.

Duplicate copies must not be counted as independent corroboration. A SHA-256 hash may assist identification and integrity checking. An OpenTimestamps or Bitcoin timestamp may establish that particular data existed by a particular time, but it does not independently prove that the contents are true.

2.4 Evidence-classification code

Code	Classification	Meaning
A	Authenticated documentary fact	An original record whose provenance has been adequately established.
B	Corroborated fact	Supported by independent sources.
C	Recorded allegation	A complaint, statement or report demonstrating that an allegation was made.
D	Subject testimony	An account supplied by Dr McLean without independent corroboration.

E	Reasonable inference	An interpretation supported by identified evidence.
F	Disputed proposition	Materially conflicting accounts or records.
G	Unverified allegation	Insufficient evidence currently available to establish the claim.
H	Outstanding investigative question	A matter requiring additional records, witnesses or expert examination.

Categories C to H must not be converted into category A merely through repetition, publication, archive placement, emotional intensity, spiritual language, or the passage of time.

PART III — THE IMMEDIATE HUMANITARIAN EMERGENCY

3.1 Financial deprivation

The master statement records that Commonwealth Bank blocked Dr McLean's cards, leaving him without practical access to his money. He reports that this prevented him from purchasing food, obtaining necessities and independently managing daily circumstances.

The statement demands immediate investigation of the restrictions, restoration of confirmed debit-card access, release of available funds where permitted, or an immediate transfer of $150 from account ending 0384 into a confirmed usable debit-card account, subject to verification and available funds. It further demands written reasons for the restrictions, escalation to a senior complaints officer, a complaint reference and a written response.

Classification: the blocked-card circumstance and its consequences are recorded as first-person testimony and an urgent humanitarian allegation. The bank's reasons, authorisations, legal basis and complaint handling remain outstanding investigative questions.

3.2 Food, water and basic necessities

The master statement records that Dr McLean had no food and could not wait for lengthy administrative investigation before necessities were addressed. It demands food, drinking water, access to available money, necessary medication and supplies, reliable communication and practical support.

The humanitarian principle is simple: where a person reports no food, no money access and no reliable communication, the first institutional duty is to address survival and immediate material need. Analysis, complaint handling and investigation must not become substitutes for urgent assistance.

3.3 Communication deprivation

Dr McLean reports no reliable phone credit. He states that people cannot assume he can answer calls or initiate contact, and that his inability to communicate is directly connected to his lack of financial access.

He requests communication by email, internet-based messaging, independent advocate, trusted person or another accessible method. He states that lack of phone credit must not automatically be interpreted as deliberate refusal to communicate, disappearance, incapacity or refusal of assistance.

This distinction must be preserved in all welfare, missing-person, police, hospital and provider records.

3.4 Accommodation, departure and safety

Dr McLean states that he left his accommodation because he felt trapped and believed the circumstances surrounding accommodation, financial access, support arrangements and proposed welfare intervention had become intolerable. He states that he believed he was going to die in those circumstances.

He rejects the proposition that leaving accommodation automatically establishes mental illness, incapacity or inability to make decisions. He states that he is entitled to explain his decision, have that explanation recorded accurately, and prevent his departure from being used to erase the underlying events.

The report records that Crystal is at home and safe, and that this fact should be included in welfare, accommodation and missing-person records to avoid unnecessary confusion.

3.5 Immediate protective duties

Where a person reports no food, no money access, no reliable communication and unsafe accommodation, responsible bodies should immediately:

- Provide food and drinking water.
- Restore or provide alternative access to available funds.
- Restore communication or provide an accessible alternative.
- Arrange independent disability advocacy.
- Conduct an urgent safeguarding assessment focused on present safety and material needs.
- Preserve all relevant records.

- Record the person’s stated wishes and objections.
- Explain the legal basis for any proposed compulsory action.

PART IV — SOCIAL ERASURE, ADMINISTRATIVE ANNIHILATION AND CIVIL EXCLUSION

4.1 Social erasure

Dr McLean defines social erasure as the experience in which a person's testimony is no longer heard, their character is reduced to a disputed narrative, their reputation is damaged, and their requests for help appear to disappear into administrative systems.

He states that he feels as though his testimony, character and reputation have been destroyed; that he has become a person of zero consequence whom institutions can ignore without consequence; and that his social existence and voice feel destroyed. He simultaneously declares that he is still here: his body, mind, feelings and testimony remain.

For justice purposes, social erasure must be examined as a lived consequence and potential accountability issue. It must not be dismissed as rhetorical excess, psychiatric symptom, or reason to disregard documentary evidence.

4.2 Administrative annihilation

Administrative annihilation is defined in the master statement as the progressive deprivation of the practical means to participate in ordinary life: loss of access to money, loss of reliable communication, absence of food, withdrawal or interruption of support, uncertainty about accommodation, fear of misrepresentation through welfare or missing-person processes, and difficulty obtaining independent assistance.

The cumulative result is a situation in which ordinary personal autonomy becomes difficult or impossible to exercise. The question for investigation is whether separate decisions and omissions interacted to produce this result.

4.3 Civil death as lived exclusion

Dr McLean uses "civil death" to describe the lived consequences of being unable to exercise ordinary citizenship in practice. The term should not be presented as a formal legal status unless the relevant law supports that interpretation.

The investigation must examine whether he can access his own money, obtain legal advice, make independent decisions, leave accommodation, obtain protection, maintain employment, participate in public life, have complaints properly investigated, exercise privacy and autonomy, and obtain effective remedies.

If the cumulative circumstances produce practical civic exclusion, that is a serious human-rights and administrative-justice issue even if no formal legal status called "civil death" exists.

4.4 The documentary trap

The archive advances the proposition of "the documentary trap": when an institution's own record becomes evidence. The investigation must examine whether institutions created records that contradict later explanations, demonstrate prior knowledge, reveal inconsistent classifications, or establish that a complaint was received and processed.

For every material institutional record, the investigation must ask: What did the institution know? When did it know? Who received the information? What response was given? What legal authority was relied upon? What action followed? Was the response consistent with the institution's own policies? Did the institution later give a different explanation? Is there a documentary contradiction? Did another agency rely on the earlier decision?

PART V — POLITICAL EXILE, INTERNAL DISPLACEMENT AND THE REFUGEE QUESTION

5.1 Reported political exile

Dr McLean alleges that he was effectively forced to leave Victoria and relocate to New South Wales. He describes this as political exile and contends that the departure was not simply a voluntary residential decision but occurred after prolonged alleged targeting, professional damage, financial deprivation, personal danger and failures of institutional protection.

The investigation must examine his circumstances before leaving Victoria; employment and housing history; events preceding relocation; police interactions; complaints to government agencies; reported threats and safety concerns; financial circumstances; disability-support circumstances; homelessness and periods living in his vehicle; ability to access legal representation; ability to obtain protection; consequences of relocation; responses of Victorian institutions; responses of NSW institutions; and whether intergovernmental responsibilities were properly managed.

5.2 Internal refugee terminology

Dr McLean describes himself as an internally displaced refugee. This language must be recorded accurately as his testimony and public description. It must be distinguished from the formal international legal definition of refugee, which ordinarily concerns a person outside their country of nationality who is unable or unwilling to return owing to a well-founded fear of persecution.

The investigation must identify whether the terminology appeared in an internal administrative classification, human-rights submission, asylum or protection-related document, United Nations communication, third-party description, or formal determination.

The serious human-rights question is whether an Australian citizen can establish a credible account of internal displacement, institutional abandonment and alleged persecution warranting independent scrutiny, despite remaining formally within the jurisdiction of the state whose institutions he complains about.

5.3 Democracy of violence

Dr McLean describes Australia, in his lived experience, as a “democracy of violence.” This is not a claim that every person experiences violence. It is a description of structural, administrative, financial, psychiatric, reputational and social forms of harm that may operate without a single identifiable physical assailant.

The concept must be examined through the cumulative-detriment model: financial restriction may prevent relocation; inability to relocate may prolong exposure to an environment reported as unsafe; disability may increase dependence on providers; provider disputes may disrupt continuity of care; Legal Aid restrictions may prevent challenges to administrative arrangements; psychiatric classifications may affect how complaints are received; and reputational damage may reduce access to support and employment.

PART VI — JOURNALISM, ACADEMIC LIFE, ADVOCACY AND WHISTLEBLOWING

6.1 Professional and public-interest history

Dr McLean identifies as a journalist, published author, PhD researcher, mental-health advocate, public-interest complainant, documentary participant and whistleblower.

The investigation must verify his reported PhD from Victoria University in 2020 and thesis *A Splice of My Life*; the awards and recognition associated with *Recovered, Not Cured: A Journey Through Schizophrenia*; his employment with The Age; and his account that employment ended following publicity involving the Herald Sun.

The professional, economic and reputational consequences must be examined, including any connection between media coverage, employment loss, future professional opportunities and financial detriment.

6.2 Parliament House and pre-NDIS advocacy

Dr McLean reports appearing inside Parliament House and advocating for mental-health reform before the NDIS. The investigation must identify parliamentary records, submissions, meetings and correspondence; examine the issues raised; and assess the public-interest context.

It must also examine the significance of a person who advocated for improved mental-health and disability systems later reporting dependence upon those systems and serious failures within them.

A causal connection between advocacy and later adverse treatment must not be inferred merely from chronological proximity. It must be tested through documents, decision-makers, communications, timing, stated reasons and contrary explanations.

6.3 Whistleblower status and retaliation

Dr McLean describes himself as an unprotected whistleblower. The investigation must examine whether his disclosures, public advocacy, publications, complaints or archival work influenced decisions affecting his circumstances.

Relevant evidence includes government correspondence, administrative decisions, financial restrictions, support-service decisions, accommodation events, welfare-check arrangements, communications between relevant parties, and any documented connection between public activities and adverse treatment.

Retaliation must not be assumed from adversity alone. Equally, institutional denial must not be treated as proof that retaliation did not occur.

PART VII — MEDIA REPRESENTATION, CHARACTER ASSASSINATION AND DOCUMENTARY EXPLOITATION

7.1 Herald Sun and The Age

Dr McLean alleges public character assassination and reputational damage arising from media coverage. The investigation must retrieve original articles, headlines, dates, images, quotations and editorial context.

It must examine whether coverage accurately represented him; misrepresented statements; publicised private information; used psychiatric history inappropriately; exploited his sexuality; created or amplified stigma; contributed to employment loss; affected future professional opportunities; or created identifiable financial detriment.

7.2 Gay identity and reputational weaponisation

Dr McLean identifies as gay and disabled. He alleges that his sexuality has been used to stigmatise, humiliate, discredit or disadvantage him.

He further reports being labelled a rapist or paedophile despite stating that no arrest, charge or legal process established those allegations. The exact source of every accusation must be identified. It must be established whether any allegation was formally made, investigated, substantiated, withdrawn or disproved.

An unsubstantiated accusation must not be repeated as fact. Equally, the absence of prosecution does not automatically prove an accusation was fabricated. The consequences for safety, reputation, accommodation, relationships, disability support and access to justice must be examined.

7.3 Documentary projects

Dr McLean alleges that his kindness, willingness to disclose his experiences and public mental-health advocacy were exploited through the Cunningham Dax Collection documentary project and a documentary referred to as Madame.

The investigation must identify contracts, permissions, correspondence, interviews, production records and public representations. It must determine whether his personal history was used inconsistently with agreements or his understanding of the projects.

PART VIII — REPORTED THREATS, ASSASSINATION ALLEGATIONS AND MEDICAL EVENTS

8.1 Reported assassination attempt

Dr McLean alleges an attempted assassination or coordinated event intended to physically eliminate him. This is recorded as a serious allegation requiring examination.

The investigation must establish date, location, circumstances, original reports, medical records, police records, witnesses, communications, physical evidence, subsequent investigations, and any documented government or federal-agency involvement.

He reports that a person he describes as a federal agent acknowledged that an earlier event was a close call, signed an NDA, and confirmed that a report concerning a girl had been fabricated to damage his reputation. Each assertion must be examined separately: the individual, exact words, date and documentary evidence.

The reported conversation must not be assumed to prove an assassination attempt. The absence of an official prosecution must not be assumed to disprove the allegation. The available evidence must determine what is supported and what remains unresolved.

8.2 Life-threatening injury, revival and disability

Dr McLean reports a self-inflicted injury that appeared fatal, subsequent discovery of heart failure, and revival. He reports that consequences were later minimised or whitewashed.

The investigation must examine emergency-service records, hospital records, cardiovascular findings, neurological findings, cognitive assessments, rehabilitation, disability determinations, later support needs and relevant clinical opinions.

An incomplete record is not proof of a cover-up. Medical evidence must not be disregarded because Dr McLean disputes an institutional explanation. The question is whether injuries, medical findings or functional impairments were inadequately recognised, recorded or acted upon.

8.3 V2K, surveillance and gang stalking

Dr McLean reports experiences he describes as V2K, military harassment, gang stalking, surveillance, psychological torture, coordinated harassment, sexualised harassment, continuous harassment in his residence, and psychiatric weaponisation.

These reports must be recorded accurately as testimony. Available recordings, communications, physical evidence, witnesses, provider reports and independently verifiable incidents must be examined.

It must not automatically be accepted that military, intelligence or government operations are responsible. Nor may the testimony be automatically dismissed as delusion. Observable events, recordings, interpretations, subjective experiences and independently established facts must be distinguished.

8.4 Psychiatric weaponisation

Dr McLean alleges that psychiatric history has been used inappropriately to dismiss complaints, avoid investigating allegations, discredit documentary evidence, restrict legal assistance, justify unnecessary intervention, minimise reports of abuse, undermine credibility and avoid accountability.

A psychiatric diagnosis does not establish that allegations are false. A disagreement with an assessment does not establish that the assessment was deliberately weaponised. The conduct and documentary record must determine the answer.

The investigation must examine whether a psychiatric explanation was substituted for investigation of a specific event, and whether this occurred in a manner inconsistent with procedural fairness, disability rights or professional ethics.

PART IX — NDIS, ABLE POINT, SAFEGUARDING AND CONTINUITY OF CARE

9.1 Able Point and support failure

The master statement records that Able Point staff left and that Dr McLean was left without the practical assistance he expected. It demands independent examination of who was responsible for support; when and why staff left; what alternative arrangements were made; whether requests for assistance were received; what action followed reports of no food and inability to access money; why welfare response became the apparent focus; whether safeguarding procedures were activated; and who was responsible for continuity of support.

It demands preservation of staff attendance records, communications, incident reports, support notes and relevant management decisions. It states that withdrawal or failure of support must not be obscured by subsequent descriptions of behaviour.

9.2 The request to leave and transport

Dr McLean reports that on 27 October 2026 he sought transport to visit friends and was told that the support worker lacked petrol and feared losing employment if he drove him. He asks that the relevant recording, support notes, transport records and communications be preserved and investigated.

The justice question is whether a disabled person’s request for transport and community participation was treated as a practical support matter, and whether the response was consistent with the service agreement, NDIS obligations and safeguarding duties.

9.3 Reported threat and safeguarding duties

Dr McLean raises an extremely serious matter concerning an incident in which, according to his account, a person came to his home intending to murder him and was arrested by police. He understands the matter was before Wyong Court.

He asks what police records establish; what charges or allegations exist; the current status of proceedings; what Able Point knew and when; what actions followed; whether incident-reporting, safeguarding, escalation or referral obligations were engaged; and whether any required report was not made.

These are mandatory safeguarding questions. They must be answered from police, court, provider and internal records, not from assumption or institutional silence.

9.4 Continuity of care as safety

A roster is not merely a management spreadsheet. Continuity of care can be fundamental to a participant’s safety, trust and wellbeing.

The investigation must examine whether Dr McLean received the support set out in his plan and service agreement; whether complaints were documented, escalated and addressed; whether staff had safe channels to raise concerns; and whether financial and contractual arrangements matched services actually delivered.

PART X — FINANCIAL ADMINISTRATION, BANKING AND FIDUCIARY RESPONSIBILITY

10.1 NSW Trustee and Guardian and Public Guardian

Dr McLean raises concerns about NSW Trustee and Guardian and Public Guardian arrangements, including reported restrictions on access to money and a reported communication from Philip Glass, identified as associated with the Public Guardian, that further money would be taken from his account.

The investigation must establish the relevant orders; legal basis for financial administration; scope of authority; restrictions on access to money; banking access; communication with financial institutions; expenditure approvals; requests for essential funds; access to food and medication; legal representation; housing and relocation; conflicts of interest; and review and appeal mechanisms.

10.2 Commonwealth Bank

The master statement records blocked cards, inability to access money, and a demand for restoration of access or an immediate $150 transfer from account ending 0384, subject to verification and available funds.

The investigation must distinguish a restriction imposed by legal order, a bank's internal security action, and a technical access problem. It must determine whether Dr McLean had a reasonable, accessible means of obtaining food, medicine and other necessities.

It must examine complaint correspondence, the bank's responses, escalation, complaint references, written reasons, and whether the bank considered humanitarian circumstances.

10.3 Financial benefit and entrapment

Dr McLean believes that people in his life who were supposed to support him may have financially benefited from his continued dependence, entrapment or inability to leave.

This serious allegation must be investigated through contracts, service payments, invoices, funding records, provider arrangements, financial-administration records, payment authorisations, conflicts of interest, related-party relationships and communications concerning service continuation.

Payment for legitimate support is not itself exploitation. The question is whether any financial arrangement created incentives inconsistent with autonomy, safety or best interests, and whether Dr McLean was deprived of a genuine opportunity to leave, change providers or obtain independent assistance.

10.4 Financial autonomy as a human-rights issue

Financial control must be transparent, lawful and accountable. A person must not be left without a clear explanation of decisions affecting access to money.

The relevant questions are whether restrictions were lawful, necessary, proportionate, properly reviewed, consistent with Dr McLean's rights and best interests, and whether they produced destitution, unsafe dependence or inability to leave an environment he describes as unsafe.

PART XI — POLICE, LEGAL AID AND ACCESS TO JUSTICE

11.1 Police conduct

Dr McLean alleges police complicity and failure to investigate. The investigation must examine complaints, incident reports, recorded statements, refusals to investigate, decisions not to proceed, risk assessments, referral records, police correspondence, alleged failures to preserve evidence, and alleged failures to respond to SOS requests.

Police complicity must not be assumed merely from a failure to prosecute. The investigation must examine whether specific conduct supports misconduct, negligence, procedural failure, bias, improper influence or inadequate investigation.

11.2 Legal Aid exclusion

Dr McLean reports restrictions, refusals or barriers to Legal Aid in Victoria and New South Wales. The investigation must establish applications, decisions, reasons, eligibility criteria, review options, restrictions, communications, and any relationship with Public Guardian arrangements.

It must examine whether the practical outcome is that a disabled person cannot obtain representation to challenge the very arrangements restricting his financial independence. If so, this raises a serious access-to-justice issue.

11.3 The legal profession

Dr McLean raises concerns about solicitors, barristers, legal services and Legal Aid. The investigation must examine whether advice was provided, representation withdrawn, complaints addressed, conflicts existed, decisions explained accessibly, disability accommodated, and legal rights protected.

Professional misconduct must not be assumed without evidence. Equally, professional status is not immunity from scrutiny.

PART XII — HUMAN RIGHTS, INTERNATIONAL LAW AND INSTITUTIONAL RESPONSIBILITY

12.1 International human-rights audit

The matters raised engage the International Covenant on Civil and Political Rights; International Covenant on Economic, Social and Cultural Rights; Convention on the Rights of Persons with Disabilities; Convention Against Torture; and, where applicable, the Convention Relating to the Status of Refugees.

Relevant rights include equality before the law; non-discrimination; freedom from arbitrary interference; access to effective remedies; protection of persons with disabilities; legal capacity and autonomy; protection from abuse and exploitation; freedom of expression; participation in public life; privacy; humane treatment; social security; adequate housing; access to health services; and freedom from cruel, inhuman or degrading treatment.

Treaty obligations, Australian implementation, and directly enforceable domestic legal rights must be distinguished precisely.

12.2 Institutional responsibility map

The following institutions require assessment: Prime Minister and Cabinet; relevant ministers; NDIS and NDIA; DSS; Australian Human Rights Commission; Commonwealth Ombudsman; NSW Ombudsman; Victorian Ombudsman; National Anti-Corruption Commission; Australian Federal Police; NSW Police; Victoria Police; OAIC; Comcare; WorkSafe; Federal Court; administrative review bodies; Legal Aid NSW; Victoria Legal Aid; NSW Trustee and Guardian; Public Guardian; Services Australia; hospitals and health services; banks; NDIS providers; media organisations; and legal professional bodies.

For each institution, the investigation must identify legal responsibilities; relevant decision-makers; information received; documents received; decisions made; omissions; stated reasons; contradictions; review mechanisms; identifiable detriment; and available independent oversight.

The conduct of one employee must not automatically be attributed to an entire government. Individual conduct, agency conduct, ministerial responsibility and government-wide responsibility must be distinguished.

12.3 Cumulative detriment model

The central analytical framework is the cumulative removal or restriction of the attributes necessary to live with dignity in a democracy: income, compensation, money, housing, personal safety, employment, human rights, legal rights, disability support, freedom of movement, financial independence, personal autonomy, access to justice, reputation, social participation and effective protection.

The investigation must test each proposed connection against evidence and distinguish established consequences, reasonably supported consequences, and allegations that remain unverified.

PART XIII — CONTRADICTIONS, GAPS AND UNRESOLVED QUESTIONS

13.1 Contradictions register

A dedicated contradictions register must identify conflicting agency explanations; inconsistent employment classifications; different accounts of the same event; missing records; changed reasons for decisions; incorrect dates; inconsistent descriptions of Dr McLean; and contradictions between correspondence and subsequent actions.

For every contradiction, the original source documents must be identified and their significance explained.

13.2 Outstanding investigative questions

1. What precisely caused the Commonwealth Bank card restrictions, who authorised them, and what humanitarian alternatives were considered?

2. What did Able Point know about Dr McLean's safety concerns, food insecurity, financial restrictions and request to leave, and when did it know it?

3. Were safeguarding, incident-reporting, escalation or mandatory-notification obligations engaged?

4. What police records exist concerning the reported arrest, alleged threat, welfare checks and missing-person reports?

5. What hospital, ambulance, cardiovascular, neurological and rehabilitation records exist concerning the reported life-threatening injury and revival?

6. What legal authority governs NSW Trustee and Guardian or Public Guardian involvement, and what review rights exist?

7. Were Dr McLean's disclosures, advocacy, disability, sexuality, psychiatric history or prior complaints factors in any adverse decision?

8. What media records, contracts, permissions and production materials exist concerning the Herald Sun, The Age, Cunningham Dax Collection and Madame?

9. What evidence exists concerning reported threats, V2K, surveillance, gang stalking and alleged harassment?

10. What records substantiate or contradict the reported assassination allegation and the reported federal-agent conversation?

13.3 Competing explanations

The investigation must consider competing explanations. Separate institutional failures may produce a cumulative pattern without coordination. Misunderstanding, resource constraints, administrative error, risk aversion, communication breakdown, disability-related dependence and provider limitations may each contribute.

Equally, the existence of innocent explanations does not extinguish the obligation to examine documentary contradictions, safeguarding failures, financial control, reputational harm and denial of effective remedies.

PART XIV — IMMEDIATE DEMANDS, REMEDIES AND FINAL DECLARATION

14.1 Immediate humanitarian demands

1. Provide food, drinking water and urgent practical assistance now.
2. Restore access to available funds or provide an immediate lawful alternative for necessities.
3. Restore reliable communication, including phone credit or an alternative contact method.
4. Provide independent disability advocacy and communication support.
5. Conduct an urgent safeguarding assessment focused on present safety and material needs.
6. Provide safe accommodation or a practical pathway out of an environment Dr McLean describes as unsafe.

14.2 Evidence-preservation demands

Preserve all banking, hospital, police, NDIS, provider, accommodation, welfare-check, missing-person, complaint, communication, media, legal, medical and financial-administration records.

Preserve original wording, metadata, timestamps, hashes, version histories, correspondence context and source URLs. Do not overwrite, silently edit or destroy original records.

14.3 Accountability demands

Provide written reasons for every restriction on access to money and identify who authorised it. Investigate whether disclosures, advocacy, disability, sexuality, psychiatric history or prior complaints influenced decisions. Investigate all alleged coordination, misconduct, neglect, retaliation and safeguarding failures. Record objections to unnecessary psychiatric or police intervention and explain the legal basis for any compulsory action.

Provide a written response identifying responsible persons, decisions made, evidence relied upon, remedies available and timeframes for action.

14.4 Final declaration

I exist. I have survived. I have testified. I have preserved the record. I have asked for help. I have asked for justice.

I refuse to accept social erasure as the final word on my life. I am not an administrative file, a complaint number, a roster entry, a psychiatric label or a problem to be managed. I am a human being entitled to safety, food, money, communication, dignity, autonomy and a fair investigation.

THE WITNESS REMAINS. THE RECORD REMAINS. THE EVIDENCE MUST BE EXAMINED. JUSTICE MUST BE PURSUED LAWFULLY AND WITHOUT FEAR OR FAVOUR.

Safety note: If there is immediate danger, inability to access food or medication, or fear for safety, call 000 in Australia. For urgent NDIS safeguarding concerns, the NDIS Quality and Safeguards Commission lists 1800 035 544; the Commonwealth Ombudsman lists 1300 362 072 for eligible NDIS-related complaints.